

Warts - Female (Child)

Algorithm 1348 | Child (0-17 years) | Gender at birth: Female

Definition

- Assessment of a suspected wart or verruca in children and adults, with safeguards for uncertain diagnosis, genital lesions, pregnancy, diabetes, poor circulation, and immunocompromise.
- Child patient.
- Female.

Initial assessment

1. [LOCATION] Where is the wart or verruca?
2. [DURATION] How long has it been present?
3. [YES_NO] Has it changed in colour, shape, or size, bled, ulcerated, or become painful, or is the diagnosis uncertain?
4. [OPEN_TEXT] Record age, pregnancy, diabetes, poor circulation, immune suppression, medicines, and whether a genital lesion in a child raises a safeguarding concern.
5. [CAREGIVER_OBSERVATION] What has a safe parent or caregiver observed about alertness, breathing, pain, swelling, fever, eating or drinking, sleep, play, and normal activity?

Triage and redirect questions

1. Redirect → Skin Injury

Is this an open traumatic skin wound rather than a wart or verruca?

An open traumatic wound belongs in the Skin Injury pathway.

1. Disposition level 70

Is the lesion on the face or genitals, changing, bleeding, ulcerated, large, painful, recurrent, or uncertain to be a wart; or does the patient have diabetes, poor circulation, immune suppression, pregnancy, or a genital lesion requiring age-appropriate safeguarding assessment?

These features require diagnosis and treatment selection by a clinician. Genital lesions need confidential, age-appropriate sexual-health and safeguarding assessment; the exact Qatar route is governance-required.

1. Disposition level 15

Is this a typical small wart or verruca on intact non-facial, non-genital skin in an otherwise healthy, non-pregnant patient, with none of the features above?

NHS.UK guidance describes typical warts/verrucae as manageable with pharmacy over-the-counter treatments.

Advice

Lesion requiring in-person assessment

- Use the Qatar governance-approved primary-care, dermatology, sexual-health, or safeguarding pathway. Do not label an uncertain, changing, bleeding, facial, or genital lesion as a wart by telephone. Provide privacy for adolescents when safe and follow local consent and safeguarding policy.
- Call Back If: rapid change, persistent bleeding, ulceration, spreading infection, or severe pain.

Low-risk wart care

- Do not cut, burn, pick, or share treatment devices. Ask a pharmacist to confirm an age-appropriate product and follow its label; never use wart acids on the face, genitals, broken skin, or without professional advice in pregnancy, diabetes, poor circulation, or immune suppression. Wash hands after contact and avoid sharing towels, socks, or shoes.
- Call Back If: growth, colour or shape change, bleeding, ulceration, pain, or diagnostic uncertainty; skin infection or failure to improve.

First aid

References

- NHS.UK, "Warts and verrucas", <https://www.nhs.uk/conditions/warts-and-verrucae/> (page last reviewed 25 July 2023)

Provenance

Shaped to mirror the licensed After-Hours Telehealth Triage Guidelines database structure. This is IST open-source guideline content derived from cited public sources; it is not licensed STCC content. Question.DispositionLevel=null marks a See More Appropriate Guideline redirect. Demographic variants contain only source-supported differences; identical thresholds are documented rather than fabricated. Local destinations and TelemedicineEligible values require IST clinical governance approval. UAT DATA ONLY - NOT FOR REAL-PATIENT CARE OR PRODUCTION USE.

Manifest hash: 0d7fac1a47fe924284f26f957f0a3b861f43417c17a01cb9362a072a98902d9f | Status: BLOCKED_BY_RESEARCH_GAP